M28 Morphine

Scope of Practice	EP
Classification	• Opiate Analgesic (opiate agonist)
Usual Presentation	• 10mg/1ml ampule
Mechanism of Action	• Binds with Mu opioid pain receptors producing analgesia
Indications	• Severe pain
Contraindications	• Known allergy • Respiratory depression • SBP <90mmHg
Precautions	• Respiratory distress or difficulty • Elderly >65 years old • Pediatrics age <1 year (increased risk or respiratory depression) • Morbid obesity • Moderate Hypotension / Shock
Side Effects	• Respiratory depression • Hypotension • Drowsiness / reduced level of consciousness • Rash
Adult Dose & Administration	• 2.5mg increments **IV/IO** (repeat every 5 minutes, maximum 10mg) • 2-10mg **IM** (maximum 20mg) **Dilution for IV/IO Administration** • In a 10ml syringe, draw up 9mls of normal saline • Add 1ml (10mg) Morphine • Concentration is now 1mg/1ml
Pediatric Dose & Administration	• 0.1mg/kg IV/IM/IO
Use in Pregnancy and Lactation	• Pregnancy: May cause fetal harm. Morphine should only be administered when the benefit outweighs the risk. This should be discussed with the mother and/or family wherever possible. • Morphine should not be administered in labor. Morphine crosses the placenta and may result in respiratory depression of the newborn. • Lactation: Safety has not been established. Morphine should only be administered when the benefit outweighs the risk. This should be discussed with the mother and/or family wherever possible.
Additional Information	• Morphine should be administered with caution to; patients with respiratory distress or difficulty (including chest injuries), the elderly >65 years, infants <1year and morbidly obese individuals as it may suppress respiratory drive. • Use lower, carefully titrated doses and monitor closely for respiratory depression.